

# DYNAVATE-N CREAM

MAL19950248AZ

## DESCRIPTION:

CREAM

Colour : White  
Fragrance : Baby Jo

## CONTENT:

Betamethasone 17-Valerate equivalent to Betamethasone....0.1% w/w  
Neomycin Sulphate .....0.5% w/w  
(Preservative: Chlorocresol 0.1% w/w)

## PHARMACODYNAMICS:

Corticosteroids diffuse across cell membranes and complex with specific cytoplasmic receptors. These complexes then enter the cell nucleus, bind to DNA and stimulate transcription of messenger RNA (mRNA) and subsequent protein synthesis of various enzymes thought to be ultimately responsible for anti-inflammatory effects to topical application of corticosteroid. Neomycin and other aminoglycoside antibiotics interfere with bacterial protein synthesis by binding primarily to the 30S sub-unit of bacterial ribosomes.

## PHARMACOKINETICS:

Corticosteroids in the circulation are extensively bound to plasma proteins, mainly to globulin and less so to albumin. The corticosteroid-binding globulin has high affinity but low binding capacity, which the albumin has low affinity but large binding capacity. Only unbound corticosteroid has pharmacological effects or is metabolised. Corticosteroids are metabolised mainly in the liver but also in the kidney, and are excreted in the urine. The slower metabolism of the synthetic corticosteroids with their lower protein-binding affinity may account for their increased potency compared with the natural corticosteroids.

Neomycin is poorly absorbed from the alimentary tract, about 97% of an orally administered dose being excreted unchanged in the faeces. Doses of 3 g by mouth produce peak plasma concentrations of up to 4 µg/ml and absorption is similar after administration by enema. Absorption has also been reported to occur from the peritoneum, respiratory tract, bladder, wounds, and inflamed skin. Once neomycin is absorbed it is rapidly excreted by the kidneys in the active form. It has been reported to have a half-life of 2 to 3 hours.

## INDICATION:

Inflammatory dermatoses responsive to topical corticosteroid therapy, when bacterial infection is present or suspected.

## RECOMMENDED DOSE:

Apply thinly, to the affected parts, two to three times daily.

## CONTRAINDICATION:

Contraindicated in rosacea, acne vulgaris, and peri-oral dermatitis. Primary cutaneous viral infections (e.g. Herpes simplex, chickenpox). Hypersensitivity to the preparation. Application to large areas especially during pregnancy, in the elderly and in patients with impaired renal function due to a risk of ototoxicity. It is not indicated in the treatment of primary infected skin lesions caused by infection with fungi (e.g. candidiasis, tinea), or bacteria (e.g. impetigo) primary or secondary infections due to yeast. Dermatoses in children under 1 year of age, including dermatitis and napkin eruptions. Preparations containing neomycin should not be used for the treatment of otitis externa when the ear drum is perforated, because of the risk of ototoxicity.

## WARNING AND PRECAUTIONS:

Long term continuous topical therapy should be avoided where possible, particularly in infants & children, as adrenal suppression can occur even without occlusion. The face, more than other areas of the body, may exhibit atrophy changes after prolonged treatment with potent topical corticosteroids. If applied to the eyelid, care is needed to ensure that the preparation does not enter the eye, as glaucoma might result. If used in childhood, or on the face, course should be limited to five days & occlusion should not be used. Topical corticosteroids may be hazardous in psoriasis for a number of reasons including rebound relapses, development of tolerance, risk of generalised pustular psoriasis & development of local or systemic toxicity due to impaired barrier function of the skin. Do not continue for more than seven days in the absence of clinical improvement. Extend or recurrent application may increase the risk of contact sensitisation. If bacterial infection persists, systemic chemotherapy is required. Any spread of infections requires withdrawal of topical corticosteroid therapy. Bacterial infection is encouraged by the warm, moist conditions included by occlusive dressings, and the skin should be cleansed before a fresh dressing is applied.

## DRUG INTERACTIONS:

Topically applied drugs do not generally exhibit interaction with another drug as compared to when they are systemically administered.

## PREGNANCY AND LACTATION:

The safety of the use of Dynavate-N Cream in pregnant mothers has not been established. Usage should be carried out when potential benefit outweighs the risks, applications should not be in large amounts or for prolonged periods of time. It is not known whether the active ingredients are excreted in breast milk. Problems in humans have not been documented.

## SIDE EFFECTS/ADVERSE REACTIONS:

Prolonged and intensive treatment with highly active corticosteroid preparations may cause local atrophic changes in the skin such as thinning, striae, and dilation of the superficial blood vessels, particularly when occlusive dressings are used or when skin folds are involved. As with other topical corticosteroids, prolonged use or large amounts or treatment of extensive areas can result in the sufficient systemic absorption to produce the features of hypercorticism & suppression of the HPA axis. These effects are likely to occur in infants and children, and if occlusive dressings are used. In infants the napkin may act as an occlusive dressing.

## SYMPTOMS AND TREATMENT OF OVERDOSE:

For topical application of corticosteroids, problems of overdosage are unlikely to be encountered. In the case of chronic overdose or misuse, the features of hypercorticism may appear. Stop the medication and consult the doctor immediately if any unusual serious reactions happened.

## PACKING/PACK SIZE(S):

Aluminium tubes of 15 g and 50x15 g.

**JAUHI UBAT DARIPADA KANAK-KANAK  
KEEP OUT OF REACH OF CHILDREN  
FOR EXTERNAL USE ONLY**

## MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. (65683-V)  
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